

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY IN CALIFORNIA

A California-licensed Home Care Organization (HCO) providing non-medical personal care, companion, and homemaker services — helping seniors and people with disabilities live safely at home.

A step-by-step startup and operations guide

Licensing (CDSS) · Caregivers & training · Funding & billing codes · Operations

Regulatory basis: the Home Care Services Consumer Protection Act (Health and Safety Code Sections 1796.10–1796.63, from AB 1217) and Title 22, Division 6, Chapter 8; licensed by the California Department of Social Services (CDSS), Home Care Services Bureau. Funding: private pay, Medi-Cal HCBS waivers, CalAIM Community Supports, Regional Center, VA, and long-term-care insurance.

How to Use This Guide

This guide walks you through opening and operating a non-medical home care agency in California — a Home Care Organization (HCO). An HCO provides personal care, companion, and homemaker services (help with bathing, dressing, meals, housekeeping, errands, transportation, medication reminders, and companionship) so clients can stay safely in their own homes. It is the accessible, high-demand side of home care: no Certificate of Need, no clinical staff required, and a growing market as California's senior population expands.

Important — non-medical means non-medical

An HCO is a **non-medical** business. Your caregivers (Home Care Aides) assist with daily living, companionship, and self-administered medication — they do not provide skilled nursing, therapy, wound care, or medication administration. Skilled care requires a separate Home Health Agency license from CDPH. This guide is an educational resource, not legal, tax, or billing advice — verify every step and figure with CDSS and the relevant payer before you act.

The big picture, in one paragraph

Here is the whole business in a nutshell. You license your agency with the State (CDSS) as a Home Care Organization. You hire caregivers who are background-checked, registered on the state's Home Care Aide Registry, and trained. You find clients through hospitals, senior communities, case managers, and families. And you get paid — mostly private pay to start, plus Medi-Cal waivers, CalAIM Community Supports, VA, and long-term-care insurance as you grow. License it, staff it, fill it, bill it.

The path at a glance — step by step

1. Set up the business — form your California entity, get an EIN, open a business bank account, and secure an administrative office in California.
2. Get your insurance and bond — general/professional liability, workers' compensation, and the employee dishonesty bond.
3. Complete your background check — Live Scan for owners and the administrator (H.S.C. § 1796.23).
4. Write your policies — your Title 22 policy manual, training plan, and the forms it references.
5. Apply to CDSS — submit the HCO application (HCS 200) with documentation and the fee to the Home Care Services Bureau.
6. Pass review and inspection — CDSS reviews the packet and may inspect your office; then issues your license and HCO number.
7. Hire and register caregivers — Live Scan, register each aide on the Home Care Aide Registry, TB clearance, and entry-level training before any client contact.
8. Set up operations — scheduling, EVV, documentation, supervision, and billing.
9. Line up funding and referrals — private pay, Medi-Cal waivers, CalAIM contracts, VA, and referral relationships.
10. Serve and document — assess each client, build a care plan, deliver services, and document to stay inspection-ready.

Part 1 — What This Business Is

A California Home Care Organization provides non-medical home care services — assistance with activities of daily living (bathing, dressing, grooming, toileting, transferring, mobility, and eating), and companion and homemaker services (meal preparation, light housekeeping, laundry, shopping, errands, transportation, medication reminders, supervision, and companionship) — delivered by registered Home Care Aides in clients' homes. It does not provide skilled nursing or therapy.

License: Home Care Organization (HCO), issued by CDSS Home Care Services Bureau under the Home Care Services Consumer Protection Act (AB 1217) and Title 22, Division 6, Chapter 8.

With that license you can employ caregivers and provide non-medical services for pay throughout California from your one licensed office — there's no separate license per city, though each franchise location needs its own. What you can't do is provide or advertise skilled nursing, therapy, or medication administration; the moment a client needs those, you coordinate with a licensed home health agency, a physician, or home health rather than stretching your scope. Staying clearly on the non-medical side of that line is both a legal requirement and a simpler, cleaner business to run.

Why this is a strong business to start

Non-medical home care is the accessible door into a huge market. California is the largest home care market in the nation, with roughly 6.5 million residents aged 65 and older and demand accelerating as the Baby Boomers age. Unlike skilled home health, an HCO needs no Certificate of Need and no clinical staff, and California's startup barriers — while real — are lower than the medical side. What you are building is a trusted, reliable service: safe, screened, trained caregivers who show up and help people remain in their own homes. That reliability, and your reputation, is what you are really selling.

The demographics are on your side. The 65-and-older population is the fastest-growing age group in California, most people strongly prefer to age at home rather than in a facility, and the supply of quality caregivers has not kept up with demand — which is why hospitals, families, and case managers are actively looking for reliable agencies. A well-run agency enters a market that genuinely needs it. The flip side is that it is a real business with real obligations — licensing, labor law, documentation, and payroll — so it rewards operators who build good systems and take compliance seriously, which is exactly what this package is designed to help you do.

You can start focused (a particular city, a particular niche such as dementia care or post-hospital support) and grow. Many successful agencies begin largely private-pay and add Medi-Cal waiver, CalAIM, and VA clients as they build the systems to serve them.

Who your clients are

It helps to picture who you serve. Some are seniors who want to stay in their own homes and need help with bathing, meals, and companionship a few hours a day. Some are people recovering from a hospital stay or surgery who need short-term support so they don't end up back in the hospital. Some have dementia and need supervision and a calm, familiar routine. Some are younger adults with disabilities. Each comes with a different funding path (Part 6) and a different level of need, but all need the same core thing: a dependable, screened, trained caregiver who treats them with respect. Deciding early which of these you serve best — and building your caregiver roster and marketing around them — is how you stand out in a crowded market.

Part 2 — Who Regulates You

- **CDSS, Home Care Services Bureau (HCSB)** — licenses your HCO, maintains the Home Care Aide Registry, and inspects agencies (including biennial and complaint visits).
- **CDSS Caregiver Background Check Bureau (CBCB)** — processes the Live Scan background checks for caregivers, owners, and the administrator.
- **DHCS (Department of Health Care Services)** — administers Medi-Cal, the HCBS waivers (HCBA, MSSP), CalAIM Community Supports, and EVV, if you serve Medi-Cal clients.
- **Not CDPH — that's the medical side:** the California Department of Public Health licenses skilled Home Health Agencies. As a non-medical HCO you are regulated by CDSS, not CDPH.

Keeping the regulators straight saves real time: CDSS is your licensing regulator (it decides whether you can open, registers your caregivers, and inspects you); DHCS is a payer/program body (being licensed does not enroll you in Medi-Cal, and vice versa); and CDPH is a different world entirely (skilled home health). You deal with each through a different office — which is why the contact list near the end of this guide is organized by what you need.

One distinction worth internalizing early: licensing and funding are separate tracks on separate timelines. You can hold a valid HCO license and not yet be enrolled with a single Medi-Cal program, and you can be deep in a payer

conversation while your license is still pending. Sequence them deliberately — you generally need the license first, then you pursue Medi-Cal enrollment, waiver-agency relationships, or a managed-care plan's CalAIM contract — and start the funding conversations early, because they move slower than new owners expect.

Part 3 — Business, Office, and Insurance Setup

- Form your California entity (usually an LLC or corporation) with the Secretary of State, get an EIN, and open a business bank account — keep business and personal funds strictly separate.
- Secure an administrative office in California — an HCO must maintain a physical office for records, staff management, and compliance oversight, accessible for CDSS inspection. The office address is posted publicly.
- Obtain a local business license from your city or county.
- Get your insurance and bond in place (see below) before you apply.
- Obtain a National Provider Identifier (NPI) if you will bill Medi-Cal, waivers, or CalAIM.

What each of these really means

The entity is your legal shield: form an LLC or corporation so the business holds the license and the liability, and never commingle business and personal money. The office is a real requirement, not a formality — CDSS expects a secure place for confidential records and may inspect it, and the address becomes public on the HCO registry, so a home office is possible but think about privacy. Insurance is not one policy: you need general and professional liability (commonly at least \$1,000,000 per occurrence and \$3,000,000 aggregate), workers' compensation once you have employees, and the employee dishonesty bond described next.

The employee dishonesty bond

California requires an HCO to maintain an **employee dishonesty bond of at least \$10,000**, with third-party coverage (H.S.C. § 1796.55). It protects clients against theft or fraud by staff — a real risk when caregivers work alone in clients' homes — and it must stay active the whole time you operate. (Some sources quote higher figures; \$10,000 is the statutory minimum — confirm the current requirement and what your insurer recommends.)

A note on the office: because the address is posted publicly on the HCO registry and CDSS may inspect it, think about it early. A commercial office signals professionalism to referral sources and keeps your home address private; a home office is allowed and cheaper but becomes public. Either way, you need a secure place to store confidential client and personnel records — that security is something an inspector will look at.

On insurance, don't under-buy. Work with a broker who knows home care, because standard small-business policies often miss the risks you actually carry — caregivers working alone in clients' homes, transportation of clients, and allegations of abuse or theft. You'll want general and professional liability, workers' compensation the moment you have employees, the employee dishonesty bond, and often commercial auto if caregivers drive clients. Tell your broker plainly what your caregivers will do; describing it as an ordinary office business leaves you underinsured exactly where it matters most.

Part 4 — Get Licensed: The HCO Process

1. Complete Live Scan fingerprinting and obtain background clearance for owners, any 10%-or-greater owner, and the administrator (H.S.C. § 1796.23).
2. Put your insurance and the employee dishonesty bond in place and gather your documentation (articles of incorporation, job descriptions, personnel and training policies, program description).
3. Prepare your policies and procedures and your Home Care Aide training plan (entry-level and annual).
4. Complete the HCO application (form HCS 200) and submit it with the fee to the CDSS Home Care Services Bureau.
5. Respond to any deficiency notice with corrections; CDSS may schedule an on-site inspection of your office.
6. Receive your HCO license and number once CDSS is satisfied you meet the requirements.

The numbers to budget

The HCO license fee is about **\$5,603 for the two-year period** (verify current). Add the employee dishonesty bond and insurance, business setup and the annual franchise tax, Live Scan and TB costs, and about **\$35 per aide** to register each Home Care Aide on the Registry (two years). Processing typically runs **3–6 months**. California requires no Certificate of Need.

Where your manual, handbook, and forms fit. Your Policy & Procedure Manual, your Home Care Aide Handbook and training plan, and your Forms Packet are built to the HCSCPA and Title 22 — have them ready before you apply, because your policies and training plan are part of the application, and a generic template downloaded from the internet is a common reason applications stall.

What review and inspection look at — and where agencies lose time

Give yourself time to assemble the pieces — several depend on outside parties (Live Scan results, the surety bond, insurance certificates) and can't be rushed at the end:

- Your California entity documents (articles of incorporation / organization) and EIN.
- Live Scan background clearance for owners (10%+) and the administrator.
- The employee dishonesty bond and your insurance certificates (liability and workers' comp).
- Your Policy & Procedure Manual and your Home Care Aide entry-level and annual training plan.
- Job descriptions and personnel policies (hiring practices, abuse reporting).
- A program description and the completed HCO application (HCS 200) with the fee.

CDSS reviews your application packet for completeness and may inspect your office to confirm you have secure record storage, required postings, and operational systems that match your policies. The things that most often delay a first license are avoidable: an incomplete application, missing background clearances, insurance or bond that isn't in place yet, a training plan that doesn't meet the entry-level and annual requirements, and policies that are generic rather than built to California's rules. If CDSS issues a deficiency notice, respond fully and promptly — partial responses and long gaps are what stretch a 3-month approval into a 6-month one. Walk your office and your packet with an inspector's eyes before you submit.

Part 5 — Your Caregivers: Home Care Aides

Your caregivers are the business. In California they are Home Care Aides (HCAs), and the law is specific about them:

- **Registry:** every aide must be registered on the CDSS Home Care Aide Registry before having contact with a client.
- **Background check:** Live Scan fingerprinting to the DOJ and FBI (H.S.C. § 1796.23) before client contact; a disqualifying conviction bars registration without an exemption.
- **Training:** at least five hours of entry-level training before serving clients (two hours on client rights and responsibilities, three on infection control, emergency procedures, and personal-care technique), plus five hours annually.
- **Tuberculosis:** a TB exam showing absence of active TB, from 90 days before to seven days after hire (H.S.C. § 1796.45), then typically every two years if negative.
- **Employees, not contractors:** because you must direct their training, testing, and registration, aides are employees — which brings full wage-and-hour compliance.

Training that goes beyond the minimum

The five-hour entry and five-hour annual minimums are a floor, not a ceiling, and the agencies that build a reputation train past them. The tasks that actually keep clients safe — safe transfers and fall prevention, recognizing and reporting a change in a client's condition, dementia and Alzheimer's care, professional boundaries

and abuse recognition, infection prevention, and your own documentation and EVV systems — reward real training, and clients, families, and payers can tell the difference. Good training is also a retention tool: caregivers who feel competent and supported stay. Build a training plan that meets the state minimums on paper and genuinely prepares your aides in practice, and keep the records, because your training plan is part of your license application and your training logs are something an inspector checks.

California labor law — plan for it from day one

California has some of the most protective labor laws in the nation, and home care is where they bite. You must pay at least minimum wage (the statewide minimum rose to \$16.90 on January 1, 2026, and the SB 525 health care minimum wage may apply higher in some settings), daily overtime after eight hours in a day, meal and rest breaks, paid sick leave, and reimbursement of business expenses, and you must account for travel time between clients and the special rules for overnight and live-in care under the Domestic Worker Bill of Rights. SB 1350 also extended Cal/OSHA workplace protections to domestic workers employed by agencies. Build compliant scheduling, timekeeping, and payroll from the start — wage-and-hour mistakes are a leading source of liability for home care agencies.

Recruiting and keeping caregivers

Your biggest ongoing challenge won't be finding clients — it will be finding and keeping good caregivers, because the whole industry competes for them and IHSS pulls from the same pool. Treat recruiting and retention as core operations, not an afterthought. Pay fairly and on time, schedule consistently and close to where caregivers live, train genuinely so they feel competent and valued, match them to clients they work well with, and build a culture where they're respected. Turnover is expensive twice over — in recruiting and training cost, and in the disruption to clients who lose a caregiver they trusted — so a stable, well-treated roster is both your quality edge and your margin. Career pathways (helping an aide move toward CNA or beyond) can set you apart as an employer.

It's worth being clear about what the caregiver actually does day to day, because that's the service you're selling. A typical shift blends practical help and human connection: assisting with a shower and dressing, making a meal the client can enjoy, a little light housekeeping and laundry, a medication reminder, a walk or an errand, and — not least — company and conversation for someone who might otherwise be alone all day. Good caregivers also notice things: that a client is eating less, seems more confused, has a new bruise, or is unsteady on their feet, and they report it so the agency can act. That combination of dependable hands-on help and attentive eyes is what keeps clients safe at home and what families are paying for.

Part 6 — How You Get Paid: Funding & Billing Codes

Most non-medical agencies start largely private pay and add public and insurance funding as they build the systems to bill it. Here is the honest funding picture:

Source	What it is	The reality
Private pay	The client or family pays your hourly (or live-in) rate directly under your service agreement.	The core of most non-medical agencies. You set the rate from your costs; California market rates are among the highest in the nation.
Medi-Cal HCBS waivers	Programs like the HCBA Waiver and MSSP pay for personal care and homemaker services for people who would otherwise need institutional care.	Real volume, but the HCBA Waiver is at capacity with a waitlist (since 2023); you enroll and work with the waiver agency in your area.
CalAIM Community Supports	Medi-Cal managed-care plans pay for personal care and homemaker services as a Community Support (e.g., nursing-facility diversion/transition).	The growing path. You contract with the plan; personal care/homemaker is billed under T1019 with the California U6 modifier.
Regional Center / VA / LTC insurance	Regional Center (for developmental disability), Veterans' programs (Veteran-Directed Care,	Useful supplemental payers; each has its own authorization and billing process — always ask a

Source	What it is	The reality
	Homemaker), and long-term-care insurance.	new client what coverage they have.

The billing codes (non-medical / personal care)

For Medi-Cal, waiver, and CalAIM clients, non-medical services are billed with HCPCS codes (these are Medicaid codes — there is no single national rate; California sets rates through DHCS, and the 2026 HCBS rates increased with the state minimum wage). The core codes:

Code	Service	Unit
T1019	Personal care services (ADLs)	per 15 minutes
T1020	Personal care services	per diem
S5125	Attendant care services	per 15 minutes
S5126	Attendant care services	per diem
S5130	Homemaker services	per 15 minutes
S5131	Homemaker services	per diem
S5150 / S9125	Respite care (in-home)	per 15 min / per diem

California billing specifics you must know

Under CalAIM Community Supports, DHCS requires the **U6 modifier on T1019** to identify Community Supports personal care and homemaker services. Common modifiers also flag caregiver ratios and supervision. Note that T1019 may not be used for services provided by a home health aide or CNA — those are skilled-side codes. Every 15-minute unit must be backed by documentation and, for Medi-Cal, an EVV record; reconcile units against the authorization before you bill. Confirm current codes, modifiers, and rates in the DHCS Medi-Cal provider manual and Community Supports guidance before billing.

Building your payer mix

The practical art is stacking payers as you grow. Most agencies start heavily private pay — it's simpler to bill, pays promptly, and lets you build systems and reputation — then add public and insurance funding deliberately, because each new payer (a Medi-Cal waiver, a managed-care plan's CalAIM contract, VA, a long-term-care insurer) brings its own enrollment, authorization, documentation, and billing rules that take time to set up. Don't take on a Medi-Cal client before your EVV and billing systems are ready, and don't assume a single payer will fill your schedule. A healthy agency usually blends private pay with one or two public or insurance sources, matched to the clients it serves and the systems it can actually run.

Price your private-pay rate deliberately from your real costs — caregiver wages (your largest cost, and high in California), payroll taxes and workers' comp, overtime and travel time, insurance and bond, office and software, recruiting, and a margin — not from whatever a competitor charges. Put the rate, what it includes, your minimum shift length, and your cancellation and rate-change terms clearly in the service agreement. Remember that public payers pay set rates you cannot exceed, so your private-pay clients often carry the fixed costs that make the whole agency viable; plan your mix with that in mind.

A word on the other payers you'll hear about. IHSS is California's enormous consumer-directed program, but it mostly pays individual providers the client hires directly (often family members) rather than agencies, so it's more competition for caregivers than a channel for you — though understanding it helps you talk to clients. Veterans' programs can pay for home care for eligible veterans (through Veteran-Directed Care and homemaker/home-health-aide benefits), and many families don't know to ask, so it's worth screening for. Long-term-care insurance

pays for home care under many policies, but the family usually has to file and you may need to provide documentation — a service you can offer to make yourself easier to choose. Always ask a new client what coverage they have; the answer often unlocks funding they didn't know they could use.

Part 7 — Operations: Running the Agency

Getting licensed lets you open the doors; systems keep the agency running and compliant. The core operational pieces:

- **Intake, assessment, and care plans:** assess each client's needs and home, build an individualized care plan, and match a qualified aide.
- **Scheduling and EVV:** for Medi-Cal personal care you must capture Electronic Visit Verification (CalEVV or an approved alternate) — the six data points at every visit — or claims get denied.
- **Documentation:** accurate visit notes and time in/out for every visit, reconciled before billing.
- **Supervision:** review documentation and EVV, gather client feedback, and conduct supervisory contact to keep quality up at a distance.
- **Quality and compliance:** HIPAA safeguards and business associate agreements, complaint and incident handling, and a quality program.

Think of the agency as a matching-and-reliability engine: the right aide, at the right home, at the right time, doing the right tasks, documented correctly, and paid correctly. The agencies that thrive are the ones whose systems make that happen consistently — and whose caregivers stay, because turnover is the enemy of both quality and margin.

A few systems are worth getting right from the first client. Scheduling software that handles caregiver availability, client shifts, and last-minute changes saves you from the chaos that sinks small agencies. EVV, integrated with scheduling, keeps Medi-Cal claims payable. A simple, consistent visit-note and timekeeping process gives you the documentation that supervision, billing, and inspections all depend on. And a real supervision routine — reviewing notes and EVV, calling clients, and checking in with caregivers — lets you catch a problem (a client declining, a caregiver struggling, a missed visit) before it becomes a crisis or a complaint. You don't need enterprise software to start, but you do need repeatable systems.

Privacy, complaints, and quality

Three more systems round out a compliant agency. First, privacy: if you accept Medi-Cal or insurance you handle protected health information, so you need HIPAA safeguards, a documented risk analysis, and signed business associate agreements with your scheduling, EVV, and billing vendors — missing agreements and a stale risk analysis are exactly what regulators look for. Second, complaints and incidents: give clients a clear way to raise concerns, investigate promptly, document the resolution, and treat patterns as signals to fix something. Third, a simple quality program: track the things that matter — missed and late visits, client satisfaction, caregiver retention, incidents — and actually act on what the numbers show. None of this requires a big team; it requires consistent habits, and it's what turns a scramble at inspection time into a routine.

Part 8 — Timeline, Costs, and Two Worked Examples

Rough sequence

Plan for several months: set up the business and office → get insurance and the bond → complete Live Scan → write your policies and training plan → submit the HCO application → pass review and any office inspection → receive your license (about 3-6 months) → hire, background-check, register, and train caregivers → set up operations and funding → serve clients. Verify current timeframes with CDSS.

Typical startup costs to budget

- The HCO license fee (about \$5,603 for two years) and business setup (entity, EIN, annual franchise tax, local business license).

- Insurance (\$1M/\$3M liability, workers' comp) and the employee dishonesty bond.
- Live Scan and TB costs, and about \$35 per aide to register on the Home Care Aide Registry.
- Your Policy & Procedure Manual, Home Care Aide Handbook and training plan, and Forms Packet.
- Scheduling/EVV software, office setup, and initial marketing — plus a cash reserve, because payroll runs weekly while private-pay and especially Medi-Cal payments take time to arrive.

Worked example A — a private-pay agency in the Los Angeles area

An agency opens in the Los Angeles area serving mostly private-pay clients — seniors and post-hospital clients who need help with bathing, meals, medication reminders, and companionship. It gets its HCO license from CDSS, sets up an office, and builds referral relationships with hospital discharge planners, senior-living communities, geriatric care managers, and physicians. It hires and registers Home Care Aides, trains them beyond the minimum on dementia care and safe transfers, and competes on reliability and caregiver quality. As it grows, it contracts with a Medi-Cal managed-care plan for CalAIM Community Supports clients (billing T1019-U6) and takes VA and long-term-care-insurance clients. Confirm current rates before projecting revenue.

Worked example B — an agency in the San Francisco Bay Area

The economics differ by market. In high-cost areas like the Bay Area and coastal Los Angeles, wages and rent are steep, but private-pay rates are correspondingly high and demand is intense, so a focused, service-quality-driven, largely private-pay agency can do well. In lower-cost inland areas, rates are lower but so are costs, and Medi-Cal waiver and CalAIM clients make up more of the census. Neither is better in the abstract — what matters is matching your cost structure, your service area, and your payer mix to your local market, and pricing from your actual costs.

A Bay Area agency runs the same model in a higher-cost, higher-rate market. Its private-pay rates are among the highest in the country, but so are its wage costs under California's labor rules, so disciplined scheduling (watching daily overtime and travel time) and caregiver retention are decisive. It draws referrals from the region's hospitals, senior communities, and care managers, and adds Medi-Cal waiver and CalAIM clients as it builds billing systems. The licensing, caregiver, and billing steps are identical to the Los Angeles agency; the difference is the market's rates, wages, and competition for caregivers.

In both markets, your referral sources are hospital and skilled-nursing discharge planners, senior-living communities, geriatric care managers, physicians, and families searching for reliable care. Actual revenue depends on your rates, your payer mix, and your ability to recruit and keep caregivers; confirm current figures with CDSS, DHCS, and your payers.

Getting your first clients

Referrals come from relationships, and relationships take time — start building them before you have an empty schedule. Introduce yourself and your agency to the discharge-planning teams at nearby hospitals and skilled-nursing facilities, to senior-living communities that don't provide their own care, to geriatric care managers, and to physicians who see older patients. Be specific about what makes you reliable: screened, trained caregivers, consistent scheduling, and responsiveness. Families searching online are a second channel, so a clear, honest website and good reviews matter. And because home care is a trust business, your reputation — built one well-served client at a time — becomes your best marketing.

In both examples the pattern is the same: the license, the caregiver requirements, and the billing rules don't change from market to market — only the rates, the wages, the competition, and the referral relationships do. Get the compliant core right once, and you can run it anywhere in California; what makes each location succeed or struggle is local market fit and, above all, your ability to recruit and keep good caregivers while serving clients reliably. That is the real work of the business, and it never stops being the work.

Part 9 — Common Mistakes That Delay or Sink an Agency

Most agencies that stumble don't fail on the big things — they fail on avoidable ones. Learn these before they cost you.

Nearly all of these come down to the same root causes: moving faster than your systems and clearances allow, treating compliance as paperwork rather than practice, and under-planning the cash and the labor rules. None of them are hard to avoid if you know they're coming.

- **Sending an unregistered or uncleared caregiver.** Every aide must be Live Scan-cleared and on the Home Care Aide Registry before client contact — no exceptions. This is the fastest way to a violation.
- **Using a generic policy manual.** A template that isn't built to the HCSCPA and Title 22 — or that doesn't match how you actually operate — is a common reason applications stall and inspections go badly.
- **Misclassifying caregivers as contractors.** Aides are employees; misclassification brings serious wage-and-hour and tax liability.
- **Underestimating California labor law.** Daily overtime, travel time, breaks, and live-in rules trip up new agencies constantly — build compliant timekeeping from day one.
- **Skipping or fumbling EVV.** For Medi-Cal personal care, a visit missing an EVV data point is a denied claim; set up EVV before you take your first Medi-Cal client.
- **Under-capitalizing.** You pay caregivers weekly while payments — especially Medi-Cal — lag; a thin reserve is what closes new agencies, not lack of demand.
- **Confusing non-medical with skilled.** Don't advertise or provide skilled nursing — that requires a CDPH Home Health Agency license and is a different business.
- **Weak documentation and EVV discipline.** If you can't produce the visit note and EVV record for a billed visit, you can't defend the claim in an audit — build the habit from your first visit, not after your first denial.
- **Neglecting caregiver retention.** Chronic turnover shows up as missed visits, unhappy clients, and lost referrals; treating caregivers well is a business strategy, not just kindness.

Part 10 — Your First 90 Days Operating

The pattern that works: go slow enough to do it right, but start every long-lead item immediately. Caregiver recruiting and clearance, referral relationships, and payer enrollment all take longer than you expect, so begin them the day your license is in hand — or, where allowed, before. The agencies that struggle are usually the ones that took a client they weren't staffed or set up to serve; the ones that thrive build capacity first and grow into it.

Getting the license is the start line, not the finish. Your first three months set whether the agency runs smoothly.

- **Lock your caregiver pipeline.** Recruit, Live Scan, register, TB-clear, and train aides — your ability to say yes to a client depends on having cleared, trained caregivers ready.
- **Build referral relationships.** Introduce yourself to hospital discharge planners, senior communities, and care managers — referrals come from relationships and take time to build.
- **Get your systems running.** Scheduling, EVV, documentation, timekeeping, and payroll — tested and working before volume ramps.
- **Document everything from day one.** Assessments, care plans, visit notes, training, background clearances — your first inspection looks at what you actually did.
- **Nail California labor compliance.** Overtime, breaks, travel time, and sick leave — get payroll right from the first pay period.
- **Protect your caregivers and clients.** Reliable scheduling, fair pay, and good supervision keep caregivers — and retained caregivers are what clients and referral sources notice.

Part 11 — Who to Call & What to Ask

One last thought on the first 90 days: treat every early client and every referral source as the foundation of your reputation. Home care is a word-of-mouth business, and discharge planners, care managers, and families talk to each other. An agency that is organized, responsive, reliable, and honest about what it can do earns the referrals that fill its schedule. Get your documentation and your caregiver care right while you're small, and those habits carry you as you grow.

A home care agency is mostly state-regulated, so your key contacts are statewide, with a couple of local business basics. Here is who to call. Verify current numbers before you rely on them.

A useful habit throughout: get answers in writing whenever you can. When you ask CDSS a licensing question, a city about your business license, or a Medi-Cal plan about enrollment, note the person's name and keep the email. It protects you if guidance changes or a question comes up later, and it builds the organized paper trail that makes inspections and audits go smoothly.

The statewide offices

For...	Who to contact
Licensing your HCO (application, standards, inspection, registry)	CDSS Home Care Services Bureau — through the CDSS Community Care Licensing / Home Care Services pages (cdss.ca.gov)
Caregiver background checks	CDSS Caregiver Background Check Bureau (Live Scan / Guardian)
Medi-Cal, HCBS waivers & CalAIM billing	Department of Health Care Services (dhcs.ca.gov); for the HCBA Waiver, the waiver agency in your area (HCBAAlternatives@dhcs.ca.gov · 833-388-4551)
EVV	CalEVV / DHCS Electronic Visit Verification
Reporting abuse of a client	911 if in immediate danger · your county Adult Protective Services · and report to CDSS as the HCSCPA requires
Skilled home health (the other license)	California Department of Public Health (CDPH), Licensing & Certification — if you later add skilled care

The local basics — and what to ask

① City or County Business License — “What do I need to run a home care office here?”

- “I’m opening a licensed non-medical home care agency with an administrative office at [address]. What business license or tax registration do you require?”
- “Are there any zoning or home-occupation rules if my office is in a [commercial space / my home]?”

② Your metro’s referral network — build these relationships early.

- **Los Angeles:** hospital and skilled-nursing discharge planners across the L.A. basin, senior-living communities, geriatric care managers, and Area Agencies on Aging; the sheer size of the market means focusing on a service area rather than ‘all of L.A.’
- **San Francisco Bay Area:** area hospitals and senior communities, care managers, and the higher-rate private-pay market — with intense competition for caregivers, so recruiting is as important as referrals.

How to find your own city or county’s numbers (if you’re elsewhere)

1. Search “[your city] CA business license” for your local registration.
2. Start from cdss.ca.gov (Home Care Services) for licensing and the Home Care Aide Registry — the process is statewide.
3. For Medi-Cal, identify the managed-care plans in your county and the HCBA/MSSP waiver agency serving your area through dhcs.ca.gov.

4. Build your referral list from local hospitals, skilled-nursing facilities, senior-living communities, and your county's Area Agency on Aging.

If you're outside Los Angeles or the Bay Area, the approach is identical: the license and caregiver requirements are statewide, so start from CDSS, get your local business registration, identify the Medi-Cal plans and waiver agency in your county, and build your referral list from the hospitals, skilled-nursing facilities, senior communities, and Area Agency on Aging nearest you. A smaller market can be an advantage — less competition for both clients and caregivers, and an easier place to become the agency everyone knows and trusts.

Key Terms at a Glance

The alphabet soup you'll hear, in plain language:

- **HCO (Home Care Organization):** California's non-medical home care agency license — what you're getting.
- **HCA (Home Care Aide):** your caregiver, who must be registered and background-checked.
- **Home Care Aide Registry:** the CDSS public list of cleared caregivers; aides must be on it before serving clients.
- **HCSCPA / AB 1217:** the Home Care Services Consumer Protection Act — the law that governs HCOs and HCAs.
- **CDSS vs. CDPH:** CDSS licenses non-medical home care (you); CDPH licenses skilled home health.
- **EVV / CalEVV:** Electronic Visit Verification — required for Medi-Cal personal care visits.
- **HCBS waivers (HCBA, MSSP):** Medi-Cal home-and-community-based programs that pay for personal care.
- **CalAIM Community Supports:** Medi-Cal managed-care services (personal care/homemaker, billed T1019-U6).
- **IHSS:** California's large consumer-directed personal-care program — mostly individual providers, but part of the landscape.
- **T1019:** the core personal-care billing code (per 15 minutes).

A last word on terminology, because it trips people up: 'home care' (non-medical, CDSS-licensed, what this guide covers) and 'home health' (skilled, CDPH-licensed) sound almost identical but are legally distinct businesses. Use the right words in your name, your marketing, and your conversations with referral sources, and you'll avoid both regulatory trouble and the confusion that costs you clients who needed the other kind of care.

Core Rules & References

- **Home Care Services Consumer Protection Act** — Health & Safety Code §§ 1796.10–1796.63 (AB 1217): licensing, the Home Care Aide Registry, background checks (§ 1796.23), TB (§ 1796.45), operating requirements and the bond (§ 1796.55).
- **Title 22, Division 6, Chapter 8** — the HCO regulations (the 130000-series).
- **California labor law** — Domestic Worker Bill of Rights, Wage Order 15, daily overtime and breaks, paid sick leave, SB 525 and SB 1350.
- **Rules & information online** — leginfo.legislature.ca.gov; licensing at cdss.ca.gov (Home Care Services); Medi-Cal at dhcs.ca.gov.

Your Compliance Foundation

Opening the agency is the first milestone; staying compliant with the HCSCPA and Title 22 — and ready for inspection — is the ongoing work. Three companion resources are built for that: a Policy & Procedure Manual covering the HCO licensing standards, a Home Care Aide Handbook and training plan for your caregivers, and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone CDSS expects.

Before you rely on any step in this guide

Verify current requirements, forms, fees, rates, and billing codes with CDSS and, for funding, with DHCS and each

client's payer. Remember that an HCO is non-medical (assistance, not skilled care or medication administration) and that its funding is mostly private pay plus Medi-Cal waivers, CalAIM, VA, and long-term-care insurance — confirm each client's coverage before service.